PROGESTERONE

Utrogestan

#1 GLOBAL LEADER in PROGESTERONE

Source: IQVIA MIDAS®, MAT Q2 2022, 75 countries, LEU MNF

Taking care of #EveryWoman

BESINS HEALTHCARE

Innovating for Well-being

Estradiol hemihydrate

Oestrogel

#1 GLOBAL LEADER in ESTRADIOL GEL for Menopausal Hormone Therapy

Source: IQVIA MIDAS®, MAT Q2 2022, 75 countries, LEU MNF

Women take #NoPause

BESINS HEALTHCARE
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Taking care of *#EveryWoman*

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Taking care of every woman.

Fertility

ORAL

CHRONIC AUB-O[1]

Dose	No. of Days	Cycle Days
200 mg OD (1 capsule)	**10 days**	**D17 - 26**

SECONDARY AMENORRHEA [2,3]

Dose	No. of Days	Cycle Days
400 mg OD (2 capsules)	**10 days**	**at bedtime**

VAGINAL

LUTEAL PHASE SUPPORT (LPS) AFTER IVF/ICSI [4]

200 mg (1 CAPSULE) THREE TIMES DAILY

Should be in the window* **between the evening of the day of oocyte retrieval and Day 3 post oocyte retrieval.

Should be administered* **at least until the day of the pregnancy test.

REFERENCES
1. Utrogestan 200mg Oral Soft-gel Capsule Product Insert
2. Prometrium (progesterone micronized) dosing, indications, interactions, adverse effects, and more. (2020, March 4). Retrieved February 22, 2023, from https://reference.medscape.com/drug/prometrium-progesterone-micronized-342792
3. Mayo Foundation for Medical Education and Research. (2023, February 1). Progestin (oral route, parenteral route, vaginal route) proper use. Mayo Clinic. Retrieved February 22, 2023, from https://www.mayoclinic.org/drugs-supplements/progestin-oral-route-parenteral-route-vaginal-route/proper-use/drg-20069443
4. The ESHRE Guideline Group on Ovarian Stimulation, Bosch E, Broer S, Griesinger G, et al. ESHRE guideline: ovarian stimulation for IVF/ICSI. Hum Reprod Open, 2020(2), hoaa009. https://doi.org/10.1093/hropen/hoaa009

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Taking care of every woman.

Maternity

GIVEN UP TO 16 WEEKS

Threatened/ Recurrent Miscarriage management

PRISM

PRogesterone in Sponteneous Miscarriage Trial

(UTROGESTAN is the brand used in the PRISM Trial)

PROMISE

PROgesterone in recurrent MIScarriagE trial

(UTROGESTAN is the brand used in the PROMISE Trial)

NICE National Institute for Health and Care Excellence 2021

1.5 Management of miscarriage

eshre

Recurrent Pregnancy Loss

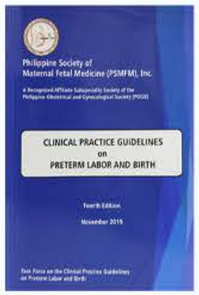
Guideline of European Society of Human Reproduction and Embryology

GIVEN UP TO 36 WEEKS

Preterm Birth prevention

FIGO International Federation of Gynecology and Obstetrics
the Global Voice for Women's Health

FIGO good practice recommendations on progestogens for prevention of preterm delivery



Philippine Society of Maternal Fetal Medicine (PSMFM), Inc.

Clinical Practice Guidelines on Preterm Labor and Birth

THREATENED/ RECURRENT MISCARRIAGE[1,2,3]

PRETERM BIRTH[4,5]

PRETERM LABOR[5]

REFERENCES:

1. Coomarasamy A, Harb HM, Devall AJ, et al. Progesterone to prevent miscarriage in women with early pregnancy bleeding: the PRISM RCT. Health Technol Assess. 2020;24(33):1-70. doi:10.3310/hta24330
2. Recommendations: Ectopic pregnancy and miscarriage: Diagnosis and initial management: Guidance. NICE. (n.d.). Retrieved February 22, 2023, from hiips://www.nice.org.uk/guidance/ng126/chapter/Recommendations#management-of-miscarriage
3. Recurrent Pregnancy Loss Guideline of European Society of Human Reproduction and Embryology. Update 2022. ESHRE Recurrent Pregnancy Loss Guideline Development Group.
4. Shennan A, Suff N, Leigh Simpson J, Jacobsson B, Mol BW, Grobman WA; the FIGO Working Group for Preterm Birth. FIGO good practice recommendations on progestogens for prevention of preterm delivery. Int J Gynecol Obstet. 2021;155:16–18. https://doi.org/10.1002/ijgo.13852
5. Philippine Society of Maternal Fetal Medicine Clinical Practice Guidelines on Preterm Labor and Birth. Fourth Edition. November 2019.

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Taking care of every woman.

Maternity

GIVEN UP TO 16 WEEKS

Threatened/ Recurrent Miscarriage management

GIVEN UP TO 36 WEEKS

Preterm Birth prevention

DUAL RISK

With early pregnancy bleeding and history of miscarriage or unexplained recurrent pregnancy loss

Dose:

400 mg (2 capsules) BID

Timing:

At the time of presentation **until 16 completed weeks of gestation**

VAGINAL

THREATENED/ RECURRENT MISCARRIAGE[1,2,3]

PRETERM BIRTH[4,5]

PRETERM LABOR[5]

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1. Coomarasamy A, Harb HM, Devall AJ, et al. Progesterone to prevent miscarriage in women with early pregnancy bleeding: the PRISM RCT. Health Technol Assess. 2020;24(33):1-70. doi:10.3310/hta24330
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Maternity

GIVEN UP TO 16 WEEKS

Threatened/ Recurrent Miscarriage management

GIVEN UP TO 36 WEEKS

Preterm Birth prevention

HIGH RISK

With prior history of Preterm Birth

Dose:
200 mg (1 capsule) OD

Timing:
16 to 36 wks

VAGINAL | ORAL (ALTERNATIVE)

SHORT CERVIX

Asymptomatic but with cervical length ≤ 25 mm

Dose:
200 mg (1 capsule) OD

Timing:
From diagnosis to 36 6/7 weeks

VAGINAL | ORAL (ALTERNATIVE)

MULTIFETAL

Asymptomatic but with cervical length ≤ 25 mm

Dose:
200 mg (1 capsule) BID

Timing:
From diagnosis to 36 weeks

VAGINAL

THREATENED/ RECURRENT MISCARRIAGE[1,2,3]

PRETERM BIRTH[4,5]

PRETERM LABOR[5]

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Taking care of every woman.

Maternity

GIVEN UP TO 16 WEEKS

Threatened/ Recurrent Miscarriage management

GIVEN UP TO 36 WEEKS

Preterm Birth prevention

ACUTE PRETERM LABOR

Combining tocolytics with 400 mg (2 capsules) Q8H

MAINTENANCE TOCOLYSIS

200mg (1 capsule) BID or TID

VAGINAL

THREATENED/ RECURRENT MISCARRIAGE[1,2,3]

PRETERM BIRTH[4,5]

PRETERM LABOR[5]

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1. Coomarasamy A, Harb HM, Devall AJ, et al. Progesterone to prevent miscarriage in women with early pregnancy bleeding: the PRISM RCT. Health Technol Assess. 2020;24(33):1-70. doi:10.3310/hta24330
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GIVEN UP TO **16** WEEKS

Threatened/ Recurrent Miscarriage management

GIVEN UP TO **36** WEEKS

Preterm Birth prevention

DUAL RISK

With early pregnancy bleeding and history of miscarriage or unexplained recurrent pregnancy loss

Dose: 400 mg (2 capsules) BID

Timing: At the time of presentation until 16 completed weeks of gestation

VAGINAL

HIGH RISK

With prior history of Preterm Birth

Dose: 200 mg (1 capsule) OD

Timing: 16 to 36 wks

VAGINAL

ORAL (ALTERNATIVE)

SHORT CERVIX

Asymptomatic but with cervical length ≤ 25 mm

Dose: 200 mg (1 capsule) OD

Timing: From diagnosis to 36 6/7 weeks

VAGINAL

ORAL (ALTERNATIVE)

MULTIFETAL

Asymptomatic but with cervical length ≤ 25 mm

Dose: 200 mg (1 capsule) BID

Timing: From diagnosis to 36 weeks

VAGINAL

ACUTE PRETERM LABOR

Combining tocolytics with 400 mg (2 capsules) Q8H

MAINTENANCE TOCOLYSIS

200mg (1 capsule) BID or TID

VAGINAL

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Taking care of every woman.

Menopause

For women using Menopausal Hormone Therapy (MHT) and still **with an intact uterus**, the recommended dose of oral micronized progesterone should be administered **for endometrial protection.**

Œstrogel®
ESTRADIOL-17ß

+

UTROGESTAN®
micronised progesterone

100mg daily
(continuous combined MHT regimen)

REFERENCES:

North American Menopause Society (NAMS),2017

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Estradiol hemihydrate
Oestrogel

#1 GLOBAL LEADER in ESTRADIOL GEL for Menopausal Hormone Therapy

Source: IQVIA MIDAS®, MAT Q2 2022, 75 countries, LEU MNF

Let life continue to bloom!

Women take #NoPause

Not likely to produce thrombotic risk stroke, and coronary artery disease[1]

Only FDA-approved transdermal Estradiol gel available in the market[2]

Safe and effective in managing symptoms of meno **Pause**[3]

How to use Oestrogel

Timing and Duration

Use in FET

REFERENCES:

1. Cobin RH, Goodman NF on behalf of the AACE Committee. Endocrine Practice 2017; 23(7): 869-880
2. FDA Verification Portal. (2021). Food And Drug Administration Philippines. https://verification.fda.gov.ph/drug_productsview.php?showdetail=&registration_number=DR-XY42910
3. NAMS 2017 Position Statement. Menopause 2017; 24(7): 728-753

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Estradiol hemihydrate Oestrogel
#1 GLOBAL LEADER in ESTRADIOL GEL for Menopausal Hormone Therapy
Source: IQVIA MIDAS®, MAT Q2 2022, 76 countries, LEU MNF

Let life continue to bloom!

Women take #NoPause

3 EASY STEPS TO USE

Estradiol hemihydrate
Oestrogel

1

Squeeze out the gel on the spatula evenly. The usual starting dose is a **single spatula measure (1.5mg estradiol) once daily.**

2

Gently spread the gel to clean, dry,intact areas of the skin (e.g. **on the abdomen, arms, shoulders, and/or inner thighs**).The area of application should be as large as possible.

3

Allow the gel to dry for 2 minutes before covering the skin with clothing.

Gel does NOT need to be massaged into skin
Gel must NOT be applied on the breasts
or mucous surfaces such as the vulval region
*If the patient forgets to apply a dose, then she should apply
it as soon as possible and apply the next dose at the normal time.

How to use Oestrogel

Timing and Duration

Use in FET

TIMING OF INITIATION

The benefits of MHT outweighs its risks in healthy postmenopausal women who initiate MHT during the "window of opportunity" in the **10 years after the menopause or before age 60 years**

International IMS Menopause Society
Promoting education and research on midlife women's health

NAMS
THE NORTH AMERICAN MENOPAUSE SOCIETY

REFERENCES
1. De Villiers TJ, Hall JE, Pinkerton JV, Pérez SC, Rees M, Yang C, Pierroz DD. Revised global consensus statement on menopausal hormone therapy. Maturitas. 2016 Sep 1;91:153-5.
2. The NA. The 2017 Hormone Therapy Position Statement of The North American Menopause Society. Menopause (New York, NY). 2017 Jul;24(7):728-53.
3. De Villiers TJ, Gass ML, Haines CJ, Hall JE, Lobo RA, Pierroz DD, Rees M. Global consensus statement on menopausal hormone therapy. Climacteric. 2013 Apr 1;16(2):203-4.
4. Baber RJ, Panay N, Fenton AI. 2016 IMS Recommendations on women's midlife health and menopause hormone therapy. Climacteric. 2016 Mar 3;19(2):109-50.

UpToDate®

The standard recommendation for duration of menopausal hormone therapy (MHT) use has been **5 years (and not beyond age 60 years)**

DURATION OF USE

REFERENCES
Martin, K. A., & Manson, J. E. (2008). Approach to the patient with menopausal symptoms.
The Journal of clinical endocrinology and metabolism, 93(12), 4567–4575. https://doi.org/10.1210/jc.2008-1272

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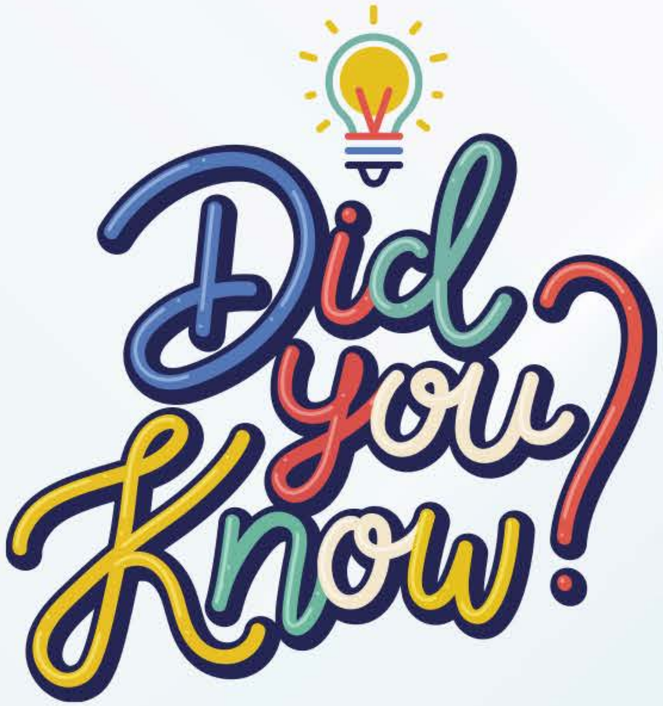

For endometrial preparation in frozen embryo transfer (FET) cycles of women at high risk for deep vein thrombosis, hyperlipdemia, and clotting disturbance you can use **OESTROGEL!**

Estradiol hemihydrate

Oestrogel

(1 spatula measure of Oestrogel = 1.5mg estradiol)

6mg (4 spatula) per day of OESTROGEL

after GnRH agonist supression[1]

Start in the first day of menstruation cycle and continued until endometrial thickness reached 8 mm.[1]

- Clinically proven (**with lower misscarraige rate and higher ongoing pregnancy and live birth rates in comparison to estradiol valerate oral tablet**)[1]
- Avoids first pass hepatic metabolism
- Convenient (easy to administer) and affordable

REFERENCES:

Tehraninejad ES, Kabodmehri R, Rashidi BH, Jafarabadi M, Keikha F, Masomi M, Hagholahi F. International Journal of Reproductive Biomedicine. 2018 Jan;16(1):51.

How to use Oestrogel | Timing and Duration | Use in FET